

How to get the most from rehabilitation

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We're smarter together

We help our clients
to help people
back to work
and better
health



Focus on rehabilitation

- Knowing when to refer: What does the evidence say
- Early intervention
- What about later on?
- Knowing when to stop

What does the evidence say?



For mild to moderate
musculoskeletal and mental
health conditions...

an early
intervention
stepped-care
approach...

is the most cost-
effective strategy
for achieving good
RTW outcomes

Early bird gets the worm.....

78% claimants
RTW within
deferred period &
83% within 1
year absence
(L&G)

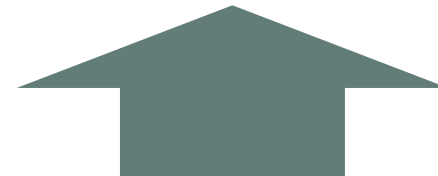
80% mental
health absences
lasted 7 months
vs 2 yrs without
rehab (Canada
Life)

Average claims
duration 6 weeks
ROI = £9 benefit
saving for every
£1 spent on rehab



Vocational rehab
evidence base

Nature of L&H
policies



Rehabilitation through the lifecycle of a claim

Within Deferred Period

- Stepped care models (mental health, musculoskeletal, cancer)
- Work focussed healthcare + accommodating workplace

Short term claims (e.g. < 2yrs)

- Proactive vocational case management (F2F or telephonic)
- Work coaching, workplace assessments / conferences
- Treatment as required (e.g. CBT, physio)

Long term claims (e.g. > 2yrs)

- Exploratory holistic assessment (strongly consider telephonic vs face to face)
- Consider full biopsychosocial evaluation & suitability for support

Tips for deciding when to engage rehab later in a claim

- Complete a full biopsychosocial review of the claim
- Weigh up the enabling factors and barriers to successful RTW with rehab versus the projected cost of a full rehabilitation intervention
- Do more to understand the psycho-social aspects:
 - Does the claimant believe that they will recover?
 - Do they believe they will be able to RTW?
 - How do they feel they are coping?
 - How confident does the claimant feel about a recovery to work?
 - What is the relationship with the workplace? = Hierarchy of RTW
 - Were there any work related factors involved in the absence?
 - What social support does the claimant have in place to support their recovery?

Knowing when to stop: considerations

- Limited objective improvements in function/work capacity with rehab
- RTW is no longer a goal e.g. after a considerable amount of time
- Repeated failed attempts to RTW (or arrange a RTW)
- Concomitant conditions are having a significant impact on recovery (e.g. addictions)
- The balance of psychosocial factors aren't positive

When in doubt consult your friendly clinician!





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